Clinical Study Report: Randomized Phase II Trial of Drug A

1. Study Design

This was a multicenter, randomized, double-blind, placebo-controlled phase II study evaluating the safety and efficacy of Drug A in patients with moderate-to-severe disease. Patients were randomized 1:1:1 to receive placebo, Drug A 10 mg, or Drug A 20 mg once daily for 12 weeks.

Table 1: Demographics and Baseline Characteristics

Characteristic	Placebo (N=100)	Drug A 10 mg (N=102)	Drug A 20 mg (N=98)
Age, mean (SD)	45.2 (11.3)	46.1 (10.9)	44.9 (11.5)
Female, n (%)	58 (58.0)	61 (59.8)	55 (56.1)
White, n (%)	82 (82.0)	85 (83.3)	79 (80.6)
BMI, mean (SD)	28.4 (4.2)	28.1 (4.5)	28.7 (4.1)

2. Efficacy Results

The primary endpoint was the proportion of patients achieving a clinical response at Week 12. Drug A demonstrated a dose-dependent improvement in response rate compared with placebo.

Table 2: Primary Efficacy Endpoint (ITT Population)

Endpoint	Placebo	Drug A 10 mg	Drug A 20 mg	p-value vs Placebo
Response rate, n (%)	25 (25.0)	42 (41.2)	48 (49.0)	<0.001
Odds ratio (95% CI)	—	2.12 (1.18-3.82)	2.89 (1.58-5.29)	—
Median time to response, weeks	8.5	6.2	5.1	<0.01

3. Safety Results

Treatment-emergent adverse events (TEAEs) were reported in all treatment groups. The majority of TEAEs were mild to moderate in severity.

Table 3: Summary of Treatment-Emergent Adverse Events

Adverse Event	Placebo (N=100)	Drug A 10 mg (N=102)	Drug A 20 mg (N=98)
Any TEAE	80 (80.0)	75 (73.5)	78 (79.6)
Headache	12 (12.0)	18 (17.6)	22 (22.4)
Nausea	8 (8.0)	10 (9.8)	14 (14.3)
Fatigue	10 (10.0)	9 (8.8)	11 (11.2)
Serious AE	5 (5.0)	4 (3.9)	6 (6.1)
Discontinuation due to AE	3 (3.0)	2 (2.0)	4 (4.1)

Conclusion: Drug A 20 mg showed statistically significant efficacy improvement with a manageable safety profile. The benefit-risk profile supports further evaluation in phase III trials.